

FC-003: Rachel Torres — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	FC-003
Persona	Family Caregiver	Next Step	Feed into `/summarize-interview` with other FC interviews

SESSION DETAILS

Date	2026-03-30	Duration	35 min
Interviewer	PM Lead	Setting	Remote, voice only
Location	Phone call (Rachel between school drop-off and mother's visit)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your role caring for your mother.

I'm the everything person. My mum Gloria is seventy-four — COPD, arthritis, anxiety, nine medications daily. She lives alone, twenty minutes from me. My brother's in London, my sister's in Auckland. They call occasionally. I visit every other day, manage all the medical appointments, coordinate the pharmacy, deal with the physio. I'm a part-time teacher so my schedule is flexible-ish, but 'flexible' just means I do two jobs instead of one.

Block 1: Falls

Q: Walk me through the last time your mother had a fall.

Six weeks ago. Bathroom. She slipped getting out of the shower and fractured her wrist. Her medical alert necklace was dead — she'd forgotten to charge it for three days. I found out because she managed to crawl from the bathroom to the hallway table, pull the phone down by the cord, and call me. Ninety minutes after the fall. The physio told me afterwards: 'The next fracture could be a hip. And that's a different conversation entirely.' I knew what she meant. A hip fracture at seventy-four with COPD — that's a hospital stay, potential pneumonia, maybe she never comes home.

[Probe] What information did you wish you had?

That the necklace was dead. Something that tells ME when the battery is low — not her, me. She forgot. She'll always forget. The system should know that and tell the person who can actually do something about it.

Block 2: Medications

Q: Tell me about managing nine medications.

Nine medications. Three times a day. Two of them can't be taken together. One has to be taken with food, one on an empty stomach. I have a university degree and I can barely keep track. I made a colour-coded chart in Excel — it takes me forty-five minutes to print and laminate a new one each week. And the pharmacy auto-refill is my most reliable tool, but it only tells me medications were picked up, not taken. The gap between dispensed and swallowed is where everything goes wrong.

Q: What would 'perfectly managed' look like?

One place where I can see: did she take her pills, has she moved around today, is anyone else checking on her today. One place, not five apps and a binder. I carry a physical binder of medical records to every appointment. The binder weighs more than my handbag. It shouldn't be this hard.

Block 3: Communication & Escalation

Q: How do you decide when something is an emergency versus watch-and-wait?

I don't know. That's the honest answer. I called 000 for the fall because obviously. But last month she had a dizzy spell and I drove over and she was fine — dehydration. Before that she was short of breath for two days and I didn't take her in because I thought it was the COPD, but the GP later said it was a mild chest infection and we should have come in sooner. I'm guessing every time. I don't know what's an emergency and what's just being seventy-four.

Block 5: Willingness to Prioritize

Q: How much time per week do you spend on care coordination?

Ten hours, easily. Plus the mental load, which never stops. Every time my phone rings after nine PM, my heart stops. Every single time. That's not a way to live. I pay sixty-five dollars a month for the medical alert necklace. Thirty a month on printed charts and binder supplies. I'd pay a hundred and eighty a month for something reliable. I evaluated a fall detection watch last year — too many false alarms, mum stopped wearing it after three weeks.

Wrap-Up

Q: Who else should I talk to?

The pharmacist at the chemist near mum's place — she knows more about mum's actual medication habits than anyone. And my friend Kate who works in aged care. She's seen the inside of the system from the other side.

PART B — Filled Note Template

Participant ID	FC-003
Persona	FC
Date	2026-03-30
Duration (min)	35
Interviewer(s)	PM Lead
KEY JOBS	Prevent another fall (next could be hip). Manage 9 medications with complex interactions. Be the communication bridge between mother, GP, physio, pharmacy. Keep own sanity as sole local caregiver.
CURRENT SOLUTION	Visits every other day. Colour-coded Excel medication chart (printed weekly, 45 min). Medical alert necklace (\$65/mo, was dead during fall). Physical binder of records carried to every appointment. Pharmacy auto-refill. Previously: fall detection watch (abandoned, false alarms). "Nine medications. Three times a day. Two can't be taken together. I have a university degree and I can barely keep track." "The gap between dispensed and swallowed is where everything goes wrong." "Every time my phone rings after nine PM, my heart stops."

WILLINGNESS	10+ hrs/week. \\$65/mo alert necklace + \\$30/mo supplies. Would pay \\$180/mo for reliable system. Rejected fall detection watch (false alarms, 3 weeks). Key: reliability over features.
EMOTIONAL INTENSITY	Rapid-fire delivery, time-pressured (between errands). Dark humour: 'The binder weighs more than my handbag.' Genuine fear about hip fracture scenario. 'Heart stops after 9 PM' said flatly, as established fact.
ESCALATION BEHAVIOR	Guessing every time. Called 000 for fall (right). Missed chest infection (wrong). Drove over for dehydration dizzy spell (overreaction). No framework for emergency vs normal aging.
SURPRISE FINDING	Pharmacy auto-refill is most reliable tool but only confirms 'dispensed' not 'swallowed.' The dispensed-to-swallowed gap is her framing of the core adherence problem. Also: battery status of alert device should notify the caregiver, not the patient.
TOOL MENTIONS	Medical alert necklace (\\$65/mo). Excel medication chart. Physical binder. Pharmacy auto-refill. Fall detection watch (abandoned). Phone calls.
REFERRAL	Local pharmacist. Friend Kate in aged care.
FOLLOW-UP ACTIONS	[] Investigate 'dispensed vs swallowed' as adherence gap framing [] Battery/device status alerting to caregiver not patient [] 'One place' dashboard concept validation